جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سعاد محمد جمعه حسان

المحافظة : القليوبيه **الحي / المدينة :** الخانكه **العنوان :** سندوه

رقم البطاقة : 26410151402269 **رقم التليفون :** 01032999304 **تاريخ تسجيل الإستمارة :** 23-10-2026

العمر(تاريخ الميلاد) : 15-10-1964 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**

التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سعاد محمد جمعه حسان
National ID: 26410151402269

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bone scintigraphy**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

[[DOCTYPE:ID_FRONT_BACK]]
National ID – Front & Back

جمهورية مصر العربية
بطاقة تحقيق الشخصية



سعاد
محمد جمعه حسان
سندوه
مركز الخانكة - القليوبية

٢٦٤ ١٠ ١٥ ١٤ ٠٢٢ ٦٩

JZ0020839

١-٢٢٩٩٩٤-٤

٢٦٤١٠١٥١٤٠٢٢٦٩ ٢٠٢٣/٠٤
بدون عمل

انثى مسلمة ارملة

١١ البطاقة سارية حتى ٢٠٣٠/٠٤/٠٥

[[DOCTYPE:OTHER_CYTOLOGY]]
CYTOLOGY

Patient No.	25713992-19	Reg. Date	04-09-2026 01:15		
Accession No.	19141562	Sex	Female	Age	62 Y
Name	سعاد محمد جمعه	Reporting Date			
Referred By	ا.د/ احمد على عاصم	Sample Collected Outside The lab			

(برجاء الاحتفاظ بالشرائح كسجل دائم لتشخيص الحاله)

CYTOLOGY REPORT

Specimen No: 12051/2026
Specimen: Right breast retro-areolar mass, U/S guided tru-cut biopsy

GROSS PATHOLOGY :

Multiple fragmented grey/white tissue cores up to 1 cm length, totally submitted.

CYTOLOGY :

Sections examined revealed cores of tumor tissue formed of nests and trabeculae of malignant epithelial cells with moderated attempts at tubular formation, moderate pleomorphism and few mitosis. The stroma showed moderate desmoplasia.

DIAGNOSIS :

Right breast retro-areolar mass, U/S guided tru-cut biopsy:
Invasive duct carcinoma, grade II.

SIGNATURE
Dr: Mostafa Fawzy, MD

P.O. :

المعامل الخارجية

Call Center 16064

.. نتائج يعتمد عليها ..

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جمهورية مصر العربية
بطاقة تحقيق الشخصية



سعاد
محمد جمعه حسان
سندوه
مركز الخانكة - القليوبية

٢٦٤ ١٠ ١٥ ١٤ ٠٢٢ ٦٩

JZ0020839

١-٢٢٩٩٩٤-٤

٢٦٤١٠١٥١٤٠٢٢٦٩ ٢٠٢٣/٠٤
بدون عمل

انثى مسلمة ارملة

١١ البطاقة سارية حتى ٢٠٣٠/٠٤/٠٥

Patient No.	25713992-19	Reg. Date	04-09-2026 01:15		
Accession No.	19141562	Sex	Female	Age	62 Y
Name	سعاد محمد جمعه	Reporting Date			
Referred By	ا.د/ احمد على عاصم	Sample Collected Outside The lab			

(برجاء الاحتفاظ بالشرائح كسجل دائم لتشخيص الحاله)

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SIGNATURE
Dr: Mostafa Fawzy, MD

P.O. :

المعامل الخارجية

Print Date: 08-09-2026 18:12

Call Center 16064
.. نتائج يعتمد عليها ..

Patient No.	25713992-19	Reg. Date	04-09-2026 01:15
Accession No.	19141562	Sex: Female	Age: 62 Y
Name	سعاد محمد جمعه	Reporting Date	
Referred By	ا.د/ احمد على عاصم	Sample Collected Outside The lab	

(برجاء الاحتفاظ بالشرائح كسجل دائم لتشخيص الحاله)

CYTOLOGY REPORT

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SIGNATURE
Dr: Mostafa Fawzy, MD

P.O. :

المعامل الخارجية

Print Date: 08-09-2026 18:12

Call Center 16064
.. نتائج يعتمد عليها ..

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	سعاد محمد جمعه حسان	**Case No.**	**406238**
Sex/Age	**Female 61 Y**	**Patient No.**	**30250**
Registered Date	**22/9/2026 09:52:45 AM**	**Referred By Dr.**	**Out Patient**
Validation Date	**12/10/2026 10:25:55 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**19/10/2026 02:41:07 PM**	**Pathology No.**	**SR-1074/26**

Pathology Report

Clinical Data: Right breast cancer.

Nature of Specimen: Block and slide revision.

Gross Examination:
One paraffin block and one H&E slide labeled (1205/26),

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
-Histologic type: Invasive duct carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2).
- Ducal carcinoma in situ: Absent.
- Microcalcification: Absent.
- Angioinvasion: Present.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results:
* IPT (All controls showed appropriate reactivity):
- ER: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong membrane staining in more than 10% of the tumor cells (score 3+).
- Ki67%: 40%.

Conclusion:
Right breast mass, Tru-cut biopsy:
-Invasive duct carcinoma, Grade 3.
IPT:
- ER: Positive (score 3/8).
- PR: Negative (score 0/8).

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي

Patient Name	سعاد محمد جمعه حسان	**Case No.**	**406238**
Sex/Age	**Female 61 Y**	**Patient No.**	**30250**
Registered Date	**22/9/2026 09:52:45 AM**	**Referred By Dr.**	**Out Patient**
Validation Date	**12/10/2026 10:25:55 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**19/10/2026 02:41:07 PM**	**Pathology No.**	**SR-1074/26**

- Her2: Positive (score 3+).
- Ki67%: 40%.

***Scoring system: "ER, PR"*
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.** *Scoring system: "Her2"*
Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghonaim, MD.

Medical No. : 10381
Patient's Name: Soad Mohammed Gomaa Hassan
Date: 2026-10-06
Referred by: Prof.Dr. **no doctor**
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Pathologically proven right breast IDC grade II, Re-assessment study.

Findings:

Right breast.

- Scattered fibroglandular densities. (ACR b)
- Upper central, middle third, irregular speculated high density mass lesion with surrounding distortion yet no suspicious microcalcifications. Few scattered punctate microcalcifications with no specific clustering as well as benign macro-calcifications.
- **Ultrasound** demonstrated 12 o'clock, 3 cm from nipple, 2.5 mm deep to the skin, irregular anti-parallel heterogenous hypoechoic solid mass lesion with indistinct margin & surrounding echogenic grin, posterior acoustic shadowing and minimal internal vascularity on color Doppler study measuring about 31 x 21 mm.
- Normal skin/nipple/areola complex.
- One level 1 indeterminate axillary lymph node is noted measuring 14 x 6 mm with cortical thickness of 2.5 mm yet focal cortical thickness is noted measuring 3 mm.
- No pathological infra or supra-clavicular lymph node.

Left breast.

- Scattered fibroglandular densities. (ACR b)
- Evidence of benign macro-calcifications.
- No masses speculations, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

C***onclusion:-***

Right breast (BIRADS 6): Upper central pathologically prove malignant mass lesion with ipsilateral one indeterminate axillary level 1 lymph node.

Left breast (BIRADS 2): benign calcifications.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Medical No. : 11039
Patient's Name: Soad Mohammed Gomaa Hassan
Date: 2026-10-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

جوستاف روسي انترناشونال مصر

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA.

I- CHEST:

- Right breast lesion is seen measuring 30 x 22 mm for sono-mammography.
- Multiple tiny bilateral scattered pulmonary nodules measuring 3-4 mm in diameter.
- Small right upper lobe calcified pulmonary nodule likely post granulomatous / inflammatory sequelae.
- Diffuse mosaic attenuation of both lungs ? small airway disease for clinical correlation.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- Left adnexal cyst measuring 61 mm in diameter.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **Multiple tiny bilateral scattered pulmonary nodules likely inflammatory for follow up.**
- **Otherwise no pulmonary or hepatic metastatic lesions.**

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]